

PUBLIC HEALTH STRATEGY IN THE REPUBLIC OF SERBIA 2018-2026. YEARS

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Introduction

"Healthy people, health in all policies"

Public health is the science and art of preventing disease, prolonging life and improving health through the organized efforts of society. Public health can also be considered through the structures and processes by which the health of the population is understood, preserved and improved¹.

The Public Health Strategy in the Republic of Serbia (hereinafter: the Strategy) supports the improvement of health, disease prevention and prolonging the quality of life of the population. Good health is essential for sustainable economic and social development and a basic concern in the life of every person, family and society.

The strategy is a document that establishes a general framework for actions and identifies further directions, leaving room for solving old and new challenges.

The strategy supports the fulfillment of social concern for people's health and encourages the responsibility of the state and society in ensuring the well-being of all citizens by improving health, extending the expected duration of a quality life, and preserving a healthy living and working environment. The goals are achieved through all forms of partnership for health and emphasizing the importance of a comprehensive approach through interdisciplinarity and multisectoral cooperation.

The Strategy defines the areas of action in public health, the framework, mission, vision and principles of the Strategy, general and specific goals, action plan for action, as well as the method of implementation, monitoring, evaluation and reporting on the implementation of the Strategy.

1. Fields of action in public health

In accordance with the law governing public health, the areas of public health action are:

- 1) physical, mental and social health of the population;
- 2) health promotion and prevention of diseases and injuries;
- 3) environment and population health;
- 4) working environment and population health;
- 5) organization and functioning of the healthcare system;
- 6) handling in crisis and emergency situations.

The strategy is primarily based on action within the framework of health promotion and disease and injury prevention.

Health promotion is understood, in accordance with the definition given in the Health Promotion Charter, Ottawa 1986: "The process of enabling people to increase control over their health and improve it." The basic directions of health promotion are:

- 1) advocating for health, in order to obtain support to create essential conditions for health;
- 2) enabling people to reach their full health potential and equality in health (equal access to information, skills for a healthy life and opportunities to make healthy choices);
- 3) mediation between the various interests of society in the pursuit of health (multisectoral, interdisciplinary in all spheres of life - between individuals, families, communities, associations of citizens, the health system and other factors of society).

Prevention means reducing the risk of the occurrence of a pathological process, disease, injury, disability or any other unwanted condition or phenomenon, through activities and measures related to the improvement and preservation of health, as well as the establishment of health that has been impaired and the prevention or delay of the unfavorable course and outcome of irreversible health disorders. Primary prevention of diseases and injuries means measures for their prevention and control, which can be general (increasing standards, environmental protection and healthy behavior) and specific (such as immunization). Secondary disease prevention measures are early disease detection (screenings, self-examinations and preventive examinations) and timely treatment. It is important to mention the levels of primordial prevention (which includes the elimination of predisposing risk factors), tertiary (measures of physical and psycho-social rehabilitation and prevention of complications of diseases and injuries) and quaternary prevention (measures for "harm reduction"). Prevention can be viewed through measures of universal (interventions aimed at the population), selective (interventions aimed at vulnerable groups) and indicated prevention (interventions aimed at individuals).

2. Framework of the Public Health Strategy

The framework of the Strategy was defined in consideration of public health challenges in the Republic of Serbia, as well as the international approach presented in public health policies and areas of public health action in Europe. Focusing attention on achieving equality in health today is the basis and is linked to sustainable development through good governance in the three most important aspects of sustainable development: economic development, broad social inclusion and environmental sustainability². The European policy "Health 2020"³ highlights the values of action to achieve good health, which include equality, sustainability, quality, transparency, responsibility, gender equality, dignity and the right to participate in decision-making.

2.1. Public health challenges in the Republic of Serbia

Public health challenges in the Republic of Serbia are numerous and are linked to the consequences of the economic crisis, population migration and growing inequalities, and are viewed through health and determinants of health, the public health system and public health policy.

2.1.1. Health status and determinants of health

Health status indicators and health determinants refer to the following:

- 1) reduction in the number and aging of the population are the main features of the demographic situation in the Republic of Serbia;
- 2) a large number of residents of the Republic of Serbia fall ill, die prematurely or are disabled due to preventable diseases and injuries;

3) leading diseases and injuries are related to social and economic determinants of health, that is, they are disproportionately present in the poor and vulnerable population, which contributes to inequalities in health.

The demographic situation in the Republic of Serbia is characterized by an aging population and a negative natural increase. According to the existing demographic structure, the population of the Republic of Serbia is among the oldest in Europe (average age is 42.9 years). The existing depopulation (negative rate of natural increase -5.1/1000 in 2016), which is also contributed to by the declining fertility rate, as well as women giving birth at an increasingly later age (average number of years of life of first-time women 28.3 years) leads to a decrease of the total population of the Republic of Serbia. The average number of household members is 2.94 -⁵.

The poverty risk rate in 2016 was 25.5%. The most exposed at risk of poverty are children and young people, multi-member households and unemployed persons⁶. The unemployment rate of persons of working age in the Republic of Serbia in the third quarter of 2017 was 13.5%⁷.

The value of the gross domestic product (GDP) per capita in 2017 was EUR 5,226, while the real GDP growth rate was 1.9%⁸. Total expenditure on health care has a relatively high percentage of GDP (increase from 8.13% in 2003 to 8.98% of GDP in 2016). The share of public expenditures in total expenditures for health care decreased from 70.9% in 2003 to 58.0% in 2016, while in the same period the share of private expenditures in total expenditures for health care increased from 29.1% to 42.0%⁹.

In the period from 2002 to 2016, life expectancy at birth increased for men from 69.7 to 73.0 and for women from 75.0 to 78.0 years. The infant mortality rate decreased from 10.1 in 2002 to 5.4 in 2016, while this indicator decreased in the Roma population in the period 2005-2014. from 25.0 to 12.8. The mortality rate of children under the age of five decreased from 11.5 in 2002 to 6.1 in 2016, while in the Roma population, the value of this indicator was halved in the period 2005-2014. (from 28.0 to 14.4) ¹⁰, ¹¹.

Chronic non-communicable diseases and associated risk factors contribute to the greatest disease burden. Diseases of the heart, blood vessels and malignant tumors accounted for over two thirds of all causes of death in 2016 in Serbia. More than half of all deaths (51.7%) were the result of dying from diseases of the circulatory system, and almost every fifth person who died (21.3%) was a victim of a malignant tumor. 2.8% of the population of the Republic of Serbia died from injuries and poisoning, 3.1% from complications of diabetes and 2.6% from obstructive lung diseases¹². By comparing the standardized mortality rates for the leading causes of death in the Republic of Serbia in relation to the European country with the lowest rate and the average rate in the European Union, (hereinafter: EU) for ages 0-64, per 100,000 inhabitants, in 2014 - noticeable is that the values are highest in the Republic of Serbia: ischemic heart disease (rate of 25.6 in the Republic of Serbia, against 6.6 in Israel and 15.8 - average in the EU), cerebrovascular disease (rate of 17.3 in the Republic of Serbia, versus 2.1 in Luxembourg and 7.2 - average in the EU) and malignant neoplasms (rate of 101.6 in the Republic of Serbia, versus 44.2 in Finland and 66.7 - average in the EU). Similarly, the standardized mortality rate for leading chronic non-communicable diseases in the population aged 30-69, per 100,000 inhabitants, in 2012 was the highest in the Republic of Serbia (498.8), more than twice as high as in the country with the lowest value of this indicator. - Israel (189.9), and higher than the average values for the European Region of the World Health Organization (hereinafter: WHO) (405.0) and the EU (289.2).¹³

Smoking is still one of the leading risk factors with a trend of increasing frequency in the period from 2006 to 2013 by 3.0% (in women by 3.4% and in men by 1.9%). According to health research data, 46.1% of the population did not consume alcohol in 2013, and the use of alcohol as a daily occurrence was present in 4.7% of the population, which represents an increase compared to 2006 (3.4%). More than half of the adult population (56.3%) is overnourished, of which 21.2% are obese. This represents a significant increase compared to 2006 (17.3%). In the same period, the frequency of obesity in children almost doubled, and in 2013 it was registered in 4.9% of children¹⁴.

2.1.2. Public health system

The public health system provides and supports a wide range of activities and services, the implementation of which carries out the main tasks of public health.

The Ministry of Health is responsible for the organization of the public health system in the Republic of Serbia. The ministry responsible for education, science and technological development, the ministry responsible for agriculture, environmental protection, the ministry responsible for labor, employment, veterans' and social affairs, the ministry responsible for youth and sports have part of the responsibility for public health. the ministry in charge of culture and information, the ministry in charge of justice, the ministry in charge of finance, the ministry in charge of internal affairs, etc.

Health institutions in the public health system are defined by the Regulation on the Plan of the Network of Health Institutions¹⁵. The regulation defines 26 institutes and institutes for public health as carriers of activities in the fields of public health action in accordance with the law governing public health. A significant role in the public health system is also played by health centers in the territory for which they were established. Their network consists of 158 institutions in the Republic of Serbia. Bearing in mind certain areas of public health action, other participants outside the health care sector are also important for the outcomes, in accordance with the law governing public health.

The field of public health is regulated by the Law on Public Health and a large number of legal and sub-legal regulations. In accordance with Article 19 of the Law on Public Health ("Official Gazette of the RS", number 15/16), funds for activities in the field of public health are provided from the budget of the Republic of Serbia, autonomous provinces and local self-government units, from health insurance organizations, through the sale services and products in the public health system, performing scientific research and educational activities, from legacies, gifts, bequests and from other sources in accordance with the law.

2.1.3. Public health policy

Public health policy is focused on the social and economic determinants of health and the reduction of inequality in health and implies health care in all policies in accordance with the law regulating public health. It is based on the universal right of every individual to reach their full health potential, taking into account a holistic approach to health and viewing it through the concept of physical, mental and social well-being. In this way, the determinants of health and health as an imperative for the development of every society are in focus. This determination emphasizes the responsibility of the state, that is, the participation of all sectors of society in providing conditions that enable and support the achievement of the highest level of health for each individual. The basic values and principles of comprehensive policies are the equality of all citizens, dignity and solidarity, along with the responsibility of society, communities and individuals, their participation and transparency in decision-making¹⁶. Effective implementation of such policies requires comprehensive and synergistic management for health, with the development of models and instruments that enable better use of resources, participation of all actors and delegation of responsibility for health, with the aim of reducing inequality in health and social exclusion, as well as improving the well-being of the population.

The concept of "Health in all policies" is an approach that has been recommended as a response to the challenges brought about by the development of intersectoral cooperation and the inclusion of all subjects of society. It is an integrated approach that enables the achievement of social goals of all government departments¹⁷. Intersectoral, government-coordinated policy, common goals and integrated responses to existing problems and challenges, partnership with the non-governmental and private sector is the basis of joint management, i.e. effective implementation of health in all policies. The health sector has a leading role in working with other sectors and directly supporting the Government in developing relevant policies and achieving their goals. Successful implementation of this concept requires an institutionalized process of intersectoral problem solving, determination of mandates and responsibilities, initiatives, funding, budgets and sustainable mechanisms for the work of government institutions in the field of public health.

2.2. International approach

The strategy relies on the document "Health 2020: a European policy framework supporting actions of all levels of government and society for health and well-being"¹⁸, which was adopted on the basis of resolution EUR/RC62/Conf.Doc./8 of the WHO Regional Office for Europe. The common goals are: significantly improving the health and well-being of the population, reducing health inequalities, strengthening public health and ensuring "people-centred" health systems that are universal, equitable, sustainable and of high quality.

The "Health 2020" policy is based on four priority areas for action:

- 1) investing in health throughout the entire life cycle and empowering the population;
- 2) focus on non-communicable and infectious diseases that contribute to the greatest burden on the population in Europe;
- 3) strengthening "people-oriented" health systems, public health capacity and preparedness for emergency situations, monitoring and response;
- 4) creating resilient communities and supportive environments.

The formulation of the Strategy in this area is aligned with ten recognized key areas of public health action (10 Essential Public Health Operations and Services in Europe/10 EPHO's), in accordance with the recommendations of the WHO Regional Office for Europe¹⁹, namely:

- 1) supervision over the health and well-being of the population;
- 2) monitoring and response to health hazards and emergency situations;
- 3) health protection, including the safety of the environment and working environment, food and others;
- 4) health promotion, including activities aimed at social determinants of health and inequalities in health;
- 5) prevention and suppression of infectious and non-infectious diseases, including early detection,
- 6) ensuring good governance for health and well-being;
- 7) provision of competent public health personnel;
- 8) ensuring a sustainable organizational structure and financing;
- 9) advocacy, communication and social mobilization for health;
- 10) promotion of public health research for evidence-based policy and practice.

A very important document for the projection of the desired outcomes in public health by 2020 is also the third program of the EU for activities in the field of health 2014-2020. year²⁰, which focuses on the following basic goals²¹:

- 1) health promotion, disease prevention and strengthening of the environment that supports healthy lifestyles;
- 2) protection of citizens from serious cross-border threats;

- 3) contribution to innovative, efficient and sustainable health systems;
- 4) encouraging access to better and safer health care for citizens.

Current national strategies of other countries (Republic of Finland, England, Northern Ireland, Federal Republic of Germany, Republic of Croatia, Australia, etc.) represent valuable help.

The strategy is fully based on the United Nations Resolution "Transforming our world: Agenda for sustainable development until 2030." and intended goals²².

3. Mission, vision and principles

The Strategy's mission is to develop and implement a system of knowledge, skills and activities aimed at improving health, preventing and suppressing diseases, prolonging and improving the quality of life through organized measures of society.

The vision of the Strategy is healthier people, reduced inequalities in health and joint actions of government and society for health and well-being.

The principles of the Strategy are based on human rights and are:

- 1) responsibility of the state and society for health;
- 2) all forms of partnership for health improvement;
- 3) focus on the population and social groups;
- 4) orientation towards local self-government;
- 5) intersectoral and multidisciplinary work;
- 6) actions for social determinants, risk factors and reduction of inequalities in health.

4. General and specific objectives of the Strategy

Following the mission, vision and principles of public health, the Strategy has the following general goals, which include specific and operational goals:

- 1) improving health and reducing inequality in health;
- 2) improvement of the environment and working environment;
- 3) prevention and suppression of diseases and leading risks to the health of the population;
- 4) development of health promotion actions in the community;
- 5) support for the development of accessible, high-quality and efficient health care;
- 6) development of a public health system based on research evidence;
- 7) improvement of management, communication and partnership for the implementation of the principle "health in all policies".

4.1. Improving health and reducing inequalities in health²³

4.1.1. Improving monitoring and evaluation of health status and health inequalities

4.1.1.1 All local self-government units/LGUs (municipalities and cities) through their health councils, will report and publish annual analyzes of health status based on indicators of health statistics, indicators of the state of the living and working environment, demographic and socioeconomic determinants of health;

4.1.1.2 A system of monitoring the health status of the population of the Republic of Serbia will be established through special research, in accordance with previous practice, the plan of the WHO and the EU;

4.1.2 Improving nutrition and physical activity of the population of the Republic of Serbia (so that the share of the adult population and those under the age of 18 who eat properly and are physically active increases by 10%;

4.1.2.1 Adoption of the Nutrition and Physical Activity Improvement Strategy (SUIF);

4.1.2.2 Adoption of the National Program for the Improvement of Food Availability;

4.1.2.3 Adoption of the National Food Safety Improvement Program;

4.1.2.4 Implementation and monitoring of the National Program for the Prevention of Obesity in Children and Adults;

4.1.3 Prevention and suppression of smoking and exposure to tobacco smoke, harmful use of alcohol and drug abuse (so that the proportion of the adult population and those under the age of 18 who smoke daily or occasionally, are exposed to tobacco smoke in the work environment and in public places, drink excessively alcohol and drug abuse reduced by 10%;

4.1.3.1 Support for the implementation of the Tobacco Control Strategy and the Action Plan for its implementation is provided continuously;

4.1.3.2 Supporting the work of established intersectoral bodies important in tobacco control, continuously;

4.1.3.3 Support the change of existing or the adoption of new regulations until 2026, in accordance with the recommendations of the Guide for the implementation of the WHO Framework Convention on Tobacco Control and the legislation of the European Union;

4.1.3.4 Implementation and monitoring of the National Program for the Prevention of Harmful Use of Alcohol and Alcohol-Caused Disorders in the Republic of Serbia;

4.1.3.5 Continuous support for initiatives of local self-government units to limit the availability of alcoholic beverages;

4.1.3.6 Education of employees in primary health care to recognize persons with the problem of excessive alcohol use and the application of short intervention methods, will be carried out continuously;

4.1.3.7 Support for the implementation of the Strategy for the Prevention of Drug Abuse for the period 2014-2021. years, continuously;

4.1.4 Improving the health of the population of the Republic of Serbia in areas of leading public health importance;

4.1.4.1 Implementation and monitoring of the National Program for the preservation and improvement of the sexual and reproductive health of the citizens of the Republic of Serbia, as well as the support of the Strategy and encouragement of childbirth;

4.1.4.2 Continuous support for the implementation and evaluation of activities in the field of early development;

4.1.4.3 Improvement of preventive dental health care, implemented continuously;

4.1.4.4 Support for the adoption of the Mental Health Care Development Strategy for the period after 2018 and the Action Plan for its implementation;

4.1.4.5 Continuous support for the implementation and evaluation of the Strategy for the prevention and suppression of violence against women and children in the family and in partner relationships;

4.1.5 Improving the health of vulnerable social groups through the development and implementation of additional measures to reduce inequality in health (so that the share of vulnerable groups that assess their health as very good and excellent increases by 10%);

4.1.5.1 Defining additional health care measures intended for priority sensitive social groups on the territory of local self-government units/LGUs, continuously;

4.1.5.2 Improving the accessibility of health services and the availability of health care to vulnerable social groups in the territory of local self-government units, continuously;

4.1.6 Monitoring and evaluation of the health status of the working population and inequalities in health;

4.1.6.1. Analysis of the health status of the working population on the basis of examination of harmful factors from the workplace and from the work environment and performed preventive medical examinations, in accordance with current EU regulations and directives;

4.1.6.2. Special research of sensitive groups of the working population (young people, women in the reproductive period, older workers, persons with disabilities, unemployed, employees of small and micro legal entities, farmers).

4.2. Improvement of the environment and working environment

4.2.1. Monitoring, evaluation and improvement of the environment and risk assessment for the health of the population

4.2.1.1. All local self-government units (municipalities) will report and publish annual reports on indicators of the state of the environment until 2021;

4.2.1.2. An integrated system of environmental monitoring, evaluation and improvement will be established through strategic documents, in accordance with previous practice, special research, adopted strategies, WHO and EU plans;

4.2.1.3. The National Program for Environmental Risk Assessment on Population Health will be adopted;

4.2.1.4. Providing a favorable environment for the active implementation of accepted international obligations in the field of environment and health (IEA - declarations, protocols, action plans, etc.);

4.2.1.5. Active implementation of accepted international obligations in the field of environment and health (IEA - Declarations, Protocols, Action Plans, etc.);

4.2.2. Improvement of monitoring of harmful factors from the workplace and detection of health risks

4.2.2.1. Continuous improvement of examination of harmful factors from the workplace (chemical, physical, biological, ergonomic and psychosocial harms) and annual reporting;

4.2.2.2. Timely detection of health risks related to new technologies will be carried out continuously;

4.2.2.3. Developing a database on occupational hazards and workplaces with increased risk.

4.2.3. Improving the supply of healthy drinking water

4.2.3.1. A new regulation on the healthiness of drinking water will be adopted;

4.2.3.2. The sanitary-hygienic and technical condition of the water supply system on the territory of the Republic of Serbia will be determined;

4.2.3.3. 30% of the existing small local water supply systems will be connected, primarily facilities for supplying drinking water to pre-school and school institutions, to the central water supply networks;

4.2.4. Improvement of the waste management system

4.2.4.1. The regulation in the field of waste management will be harmonized with the EU regulation;

4.2.4.2. Regional and local waste management plans will be developed;

4.2.4.3. A new national strategy for waste management will be created;

4.2.4.4. National plans for individual waste streams will be created;

4.2.4.5. The number of residents covered by organized solid waste collection will increase to 90%;

4.2.4.6. The capacities of public utility companies will be continuously strengthened in order to improve the waste management system;

4.2.4.7. A sustainable system for collecting packaging waste will be established;

4.2.4.8. The number of inhabitants living in the area covered by the sewage system will increase by 20%;

4.2.4.9. The proportion of waste water that is purified before being discharged into recipients will increase by 20%.

4.2.5. Improving the state of the environment and responding to climate change

4.2.5.1. Emissions of harmful gases from industry, home fireplaces and motor vehicles will be reduced by 20% compared to 2015;

4.2.5.2. The controlled application of plant protection products and mineral fertilizers will be continuously improved with the aim of preserving soil quality and achieving health food safety;

4.2.5.3. Strategic noise maps, action plans for noise protection and acoustic zoning of cities will be created;

4.2.5.4. Action plans will be created to respond to climate change in cities;

4.2.5.5. Action plans for improving energy efficiency will be adopted and implemented;

4.2.6. Improvement of educational activities in the field of preservation and improvement of the environment and human health

4.2.6.1. Continuous implementation of educational programs aimed at providing hygienically correct drinking water in rural areas;

4.2.6.2. Continuous implementation of educational programs on the importance of preserving and improving the state of the environment and improving energy efficiency in preschool and school institutions;

4.2.6.3. Continuous implementation of educational programs on the application of personal and collective protection measures and other measures aimed at controlling mosquitoes;

4.3. Prevention and control of diseases and leading health risks

4.3.1. Improvement of epidemiological surveillance of diseases, injuries and risk factors

4.3.1.1. Continuous improvement of registers of persons suffering from diseases and conditions of greater public health importance;

4.3.1.2. Surveillance of leading infectious diseases will be improved by 2021 and an electronic immunization register will be introduced;

4.3.1.3. Preparedness and response to emergency situations and health hazards, continuously;

4.3.1.4. The Strategy for the Development of the System for the Prevention and Control of Infectious Diseases in the Republic of Serbia will be adopted;

4.3.2. Improving the effectiveness of the system for early detection and suppression of epidemics;

4.3.2.1. The sustainability of the application of specific epidemic control measures and support will be ensured, continuously;

4.3.2.2. Standardization of laboratory procedures in the system of early detection and suppression of epidemics will be ensured;

4.3.3. Improving the prevention and control of chronic non-communicable diseases and injuries

4.3.3.1. Improvement of program health care in the field of prevention and suppression of cardiovascular diseases, continuously;

4.3.3.2. The improvement of program health care in the field of prevention and suppression of type 2 diabetes will be carried out continuously;

4.3.3.3. Improvement of program health care in the area of prevention and suppression, as well as early detection of colorectal cancer, breast cancer and cervical cancer, will be carried out continuously;

4.3.3.4. The Injury Prevention Program will be adopted;

4.3.4. Prevention and suppression of occupational diseases, work-related diseases and injuries at work

4.3.4.1. Continuous improvement of records of occupational diseases and injuries, as well as their prevention and suppression;

4.3.4.2. The Protection Program for employees in the zone of ionizing and non-ionizing radiation, especially in health institutions, will be adopted;

4.3.4.3. Continuous innovation of the plan for emergency situations due to chemical accidents in the working environment (and spreading into the environment) and due to the effects of ionizing radiation in the working and living environment.

4.4. Development of health promotion actions in the community

4.4.1. Improving the knowledge and behavior of the population in relation to preserving and improving health and reducing risk factors

4.4.1.1. Continuous improvement of the implementation of health education work in preschool institutions, primary and secondary schools until 2026;

4.4.1.2. Continuous improvement of the implementation of health education work in health institutions;

4.4.1.3. Health promotion campaigns will be implemented and evaluated in accordance with priorities.

4.4.2. Improvement of partnership and social inclusion for health in local self-government - application of mechanisms for integrated management

4.4.2.1. Health councils will be established and function in all local self-government units in accordance with the law;

4.4.2.2. All health councils will determine the proposal of the public health plan at the local level, will monitor the annual implementation of the plan through special programs in the field of public health;

4.4.3. Development and strengthening of a network of environments that support health and healthy choices;

4.4.3.1. The "Healthy Kindergarten" Program will be continuously improved with implementation in at least 45% of preschool institutions;

4.4.3.2. The "Healthy School" Program will be continuously improved with implementation in at least 30% of primary and 20% of secondary schools;

4.4.3.3. The "Healthy City" Program will be continuously improved, covering at least ten cities.

4.5. Support for the development of accessible, high-quality and efficient health care

4.5.1. Improving the availability and accessibility of health services

4.5.1.1. The current Health Institution Network Plan will be analyzed and a new one adapted to the current health needs of the population of the Republic of Serbia will be adopted;

4.5.1.2. The current Rulebook on closer conditions for the performance of healthcare activities in healthcare institutions and other forms of healthcare services will be analyzed and a new one adapted to the optimum requirements of healthcare services will be adopted;

4.5.1.3. The Plan for the Development of Healthcare Personnel in the Republic of Serbia, based on needs, will be adopted;

4.5.1.4. The basic package of health services at all levels of health care will be defined;

4.5.1.5. Funding of the basic package of health services at all levels of health care will be provided from the resources of the Republic Fund for Health Insurance;

4.5.1.6. A sustainable division of labor between health institutions will be established.

4.5.1.7. Continuous improvement of the availability and accessibility of health services for vulnerable population groups;

4.5.1.8. Equal availability and accessibility of health services for the entire population will be ensured.

4.5.2. Improving the quality of health care and patient safety

4.5.2.1. Quality standards of all levels of health care will be adopted;

4.5.2.2. A new strategy for the continuous improvement of the quality of health care and patient safety will be adopted;

4.5.2.3. A strategy for continuous improvement of the quality of health care and patient safety will be implemented, monitored and evaluated;

4.5.2.4. All health institutions will be accredited;

4.5.2.5. Procedures for respecting patients' rights will be continuously improved;

4.5.2.6. Implementation of incentive mechanisms for health service financing based on quality and efficiency criteria will begin.

4.5.3. Improving the efficiency of health care

4.5.3.1. The assessment of health technologies will be improved by including economic analysis, which will improve the decision-making process on medicines, medical devices and procedures;

4.5.3.2. A national medicine policy will be formed;

4.5.3.3. New (on average three per year) and existing good clinical practice guidelines will be continuously developed (on average two per year).

4.6. Development of a public health system based on research evidence

4.6.1. Development and empowerment of personnel in the public health system of the Republic of Serbia by improving competencies according to the European Action Plan for strengthening public health capacities and services²⁴

4.6.1.1. An analysis of public health personnel will be carried out, the Plan of needs for public health personnel in institutions from the Plan of the Network of Health Institutions will be defined and adopted;

4.6.1.2. Amendments to the Rulebook on closer conditions for the performance of healthcare activities in healthcare institutions and other forms of healthcare services will be agreed and adopted, so that among the employees, all appropriate occupations for the performance of public health tasks will be listed²⁵;

4.6.1.3. Five-year personnel training plans will be developed and adopted through accredited academic programs in public health and accredited continuing education courses in all health institutions from the Plan of the Network of Health Institutions;

4.6.1.4. Training of personnel from institutes and institutes of public health, health centers and other health institutions will be continuously carried out based on the five-year plan, and personnel

an institution outside the health care system through accredited academic programs in public health and accredited continuing education courses;

4.6.1.5. Institutions in the field of education will continuously cooperate with state administration institutions, public health institutes and institutes, other institutions and citizens' associations on the development of new competencies based on public health challenges and the needs of public health employees.

4.6.2. Development of space, equipment and information system of institutes and institutes for public health

4.6.2.1. All activities of public health institutes and institutes will be accredited;

4.6.2.2. There will be a fully developed public health information system in public health institutes and institutes;

4.6.2.3. The Institute for Public Health of Serbia will continuously develop the communication center.

4.6.3. Establishment of sustainable financing of institutes and institutes for public health and fiscal incentives for public health activities

4.6.3.1. The existing system of financing institutes and institutes of public health will be analyzed and recommendations for improvement will be defined;

4.6.3.2. Changes in the financing of institutes and institutes for public health will be adopted, so that the source of funding from the budget dominates up to 60%, through programs of general interest;

4.6.3.3. It will be proposed and adopted a minimum set of services performed by institutes and institutes for public health, financed by the Republic Health Insurance Fund, based on contributions;

4.6.3.4. Amendments to the regulations will be adopted so that the current costs of institutes and institutes for public health are covered as in all other institutions from the Plan of the Network of Health Institutions;

4.6.3.5. The financing of the activities of the Institute for Public Health of Serbia based on budget funds will be continuously improved;

4.6.3.6. Based on the analysis, the scope of incentives aimed at the public health activities of institutes and institutes for public health will be proposed to the ministry responsible for financial affairs and the Government;

4.6.3.7. Amendments to the regulations on financing public health programs on the territory of local self-government will be proposed, so that local self-government units are obliged to finance public health programs on their territory.

4.6.4. Strengthening the network of institutes and institutes for public health

4.6.4.1. A model will be developed for an operational plan for the coordination of the activities of the network of institutes and institutes for public health;

4.6.4.2. The Institute for Public Health of Serbia will implement an operational plan for coordinating the activities of the network of institutes and institutes for public health with evaluation and reporting.

4.6.5. Development of public health research and innovation

4.6.5.1. A national program for research and development in public health will be defined and adopted, with the cooperation of institutions in the field of education, institutes and institutes for public health, other institutions outside the health care system and citizens' associations;

4.6.5.2. Improvement of cooperation in research work in priority areas of public health between public health institutes, academic institutions within and outside the health sector will be carried out continuously.

4.7. Improving management, communication and partnership for the implementation of the principle of "health in all policies"

4.7.1. Development and improvement of the system for the implementation of participatory management for health

4.7.1.1. The National Council for Public Health will be functional as a mechanism of integrated management in the implementation of health in all policies;

4.7.1.2. The adoption and application of regulations in all sectors of importance for public health will be proposed.

4.7.2. Development and improvement of evidence-based informed decision-making systems

4.7.2.1. Advanced information system, decision-making based on documents;

4.7.2.2. Established system for data collection, analysis and reporting on health and determinants of health at all levels according to WHO and EU recommendations;

4.7.2.3. A system has been established to monitor the implementation of public health policies, strategies and programs at all levels, through the activities of the network of institutes and institutes for public health and the coordination of the Institute for Public Health of Serbia.

4.7.3. Improving communication about health and determinants of health

4.7.3.1. A system has been established for the implementation of a strategic approach to communication about health and determinants of health at all levels, through the activities of the network of institutes and institutes for public health.

4.7.3.2. Developed advocacy mechanisms for health at all levels.

5. Action plan for the period from 2018 to 2026

The Action Plan for the implementation of the Strategy for the period from 2018 to 2026 (hereinafter: Action Plan) describes the total range of activities undertaken in the field of public health and leaves the possibility that, in parallel with the implementation of the Strategy, the activities will be further developed, improved and evaluate.

Activities are coordinated by institutes and institutes for public health, and the implementation of activities will be carried out according to priorities and the availability of funding sources.

6. Implementation, monitoring, evaluation and reporting on the implementation of the Strategy

In the implementation of the Strategy, the ministry responsible for health affairs cooperates with all partners from the health care system, as well as with all other partners outside the health care system.

The monitoring of the implementation of the Strategy is carried out by the Institute for Public Health of Serbia "Dr. Milan Jovanović Batut", together with institutes and institutes for public health, as the bearers of activities in the fields of public health action, about which the minister responsible for health affairs is informed.

Evaluation of the implementation of the Strategy is carried out at each stage of the Action Plan implementation process. For the evaluation, the health indicators of the European Union (ECHI-2) and indicators selected by experts responsible for certain activities and programs are used. Annual reporting of the ministry

of the person responsible for health affairs is mandatory and contains: the results of the implemented activities, difficulties and problems in implementation, as well as the proposal of measures for improvement.

7. Financial resources for the implementation of the Strategy

For the implementation of this strategy, funds have been provided in the budget of the Republic of Serbia for 2018, namely:

In section 27 - Ministry of Health, within the program:

- 1801 - Regulation and supervision in the field of health, program activity 0001 - Regulation of the health system, economic classification 411 - Salaries, allowances and benefits of employees (wages), in the amount of 10,870,264 dinars and economic classification 412 - Social contributions at the expense of the employer, in the amount of 4,636,153 dinars;

- 1801 - Regulation and supervision in the field of healthcare, program activity 0007 - Administration and management, economic classification 411 - Salaries, allowances and benefits of employees (earnings), in the amount of 187,823 dinars and economic classification 412 - Social contributions borne by the employer, in the amount from 80,112 dinars;

- 1802 - Preventive health care, program activity 0001 - Support to the work of the "Dr. Milan Jovanović Batut" Institute, economic classification 423 - Contractual services in the amount of RSD 2,392,380.00 and economic classification 465 - other grants and transfers in the amount of 15,330 .315.00 dinars; program activity 0002 - Support to the work of institutes and institutes for public health, economic classification 465 - other grants and transfers in the amount of RSD 58,870,616;

- 1809 - Prevention and control of leading chronic non-communicable diseases, program activity 0001 - Support to the work of the Office for Tobacco Control, economic classification 465 - other subsidies and transfers, in the amount of 300,000 dinars.

In section 30 - Ministry of Labour, Employment, Veterans and Social Affairs, within the Program:

- 0902 - Social protection, program activity 0004 - Support to associations and local communities, economic classification 481 - Grants to non-governmental organizations in the amount of 327,883,990 dinars.

In the financial plan of the Republic Fund for Health Insurance in 2018, funds in the amount of 4,259,501,000 dinars were provided.

For the implementation of this strategy, it is not necessary to secure funds from the section of other holders or participants in the implementation of this strategy.

In 2019, the funds will be provided in accordance with the balance possibilities of the budget of the Republic of Serbia, i.e. the limits established by the Ministry of Finance.

Financial resources for the implementation of the Strategy are provided according to the activities foreseen in the Action Plan, and from the budget of the autonomous province and local self-government units, from health insurance organizations, through the sale of services and products in the public health system, through the performance of scientific research and educational activities, from legacies, gifts, donations and other sources in accordance with the law.

8. Action plan

The action plan for the implementation of the Strategy for the period from 2018 to 2026 is printed with this strategy and is an integral part of it.

9. Final part

Publish this strategy in the "Official Gazette of the Republic of Serbia".

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- ² UN. Transforming the world we live in: The 2030 Agenda for Sustainable Development General Assembly resolution adopted on 25 September 2015 http://www.un.org/ga/search/view_doc.asp?symbol=A/RES/70/1&Lang=E (accessed May 17, 2016)
- ³ WHO. Health 2020. European policy framework and strategy for the 21st century. Copenhagen: WHO Regional Office for Europe 2013.
- ⁴ Republic Institute for Statistics of the Republic of Serbia. Statistical Yearbook 2017. Belgrade: RZS 2017.
- ⁵ Republic Institute for Statistics of the Republic of Serbia. Statistical calendar of the Republic of Serbia 2018. Belgrade: RZS 2018.
- ⁶ Republic Institute for Statistics of the Republic of Serbia. Poverty and social inequality in the Republic of Serbia in 2016. Announcement, Survey on income and living conditions, number 087, year LXVII, . Belgrade: RZS 2017.
- ⁷ Republic Institute for Statistics of the Republic of Serbia. Rates of activity, employment, inactivity and unemployment.
- ⁸ Republic Institute for Statistics of the Republic of Serbia. Statistical calendar of the Republic of Serbia 2018. Belgrade: RZS 2018.
- ⁹ Selected indicators of expenditures for health care. Available at: http://www.batut.org.rs/index.php?category_id=50 (accessed May 30, 2018)
- ¹⁰ UNICEF. Research on multiple indicators of the position of women and children in Serbia, 2014. Belgrade: UNICEF 2014.
- ¹¹ Republic Institute for Statistics of the Republic of Serbia. Dev Info. Available at: <http://devinfo.stat.gov.rs/diSrbija/diHome.aspx> (accessed May 30, 2018)
- ¹² Institute for Public Health of Serbia "Dr. Milan Jovanović Batut". Health statistical yearbook of the Republic of Serbia 2016. Belgrade: IZJZ 2017.
- ¹³ European Health For All Data Base, WHO/Europe. Available at: <https://gateway.euro.who.int/en/datasets/european-health-for-all-database> (accessed 30 May 2018)
- ¹⁴ Institute for Public Health of Serbia "Dr. Milan Jovanović Batut". Research on the health of the Serbian population in 2013. Belgrade. IZJZ 2014. ¹⁵
- (¹⁶Official Gazette of RS", no. 42/06, 119/07, 84/08, 71/09, 85/09, 24/10, 6/12, 37/12, 8/14 and 92 /15).
- ¹⁶ WHO. Health 2020. European policy framework and strategy for the 21st century. Copenhagen: WHO Regional Office for Europe 2013.
- ¹⁷ WHO. European action plan for strengthening public health capacities and services. EUR/RC62/Conf.Doc./6 Rev.2: Sixty i second session of the WHO Regional Commission: Malta, 10-13. September 2012
- ¹⁸ WHO. Health 2020. European policy framework and strategy for the 21st century. Copenhagen: WHO Regional Office for Europe 2013.
- ¹⁹ WHO. European action plan for strengthening public health capacities and services. EUR/RC62/Conf.Doc./6 Rev.2: Sixty i second session of the WHO Regional Commission: Malta 10-13. September 2012
- ²⁰ European Commission. Regulation of the European Parliament and the Council on the establishment of the Health for Growth Program, third multi-annual program of the EU in the field of health for the period 2014-2020. Brussels, 9/11/2011. COM(2011) 709 final
- ²¹ The third EC health program (2014-2020).
- ²² UN. Transforming the world we live in: The 2030 Agenda for Sustainable Development. Resolution by the General Assembly adopted on September 25, 2015.
- ²³ The basis for monitoring and evaluating all specific goals are the indicators defined by the Health Survey population of Serbia.
- ²⁴ It is the seventh area of action in public health in the European Action Plan for strengthening public health capacities and service of the World Health Organization (*EPHO 7 - European Public Health Operation 7. WHO. European Action Plan for Strengthening Public Health Capacities and Services - EAP. Copenhagen: WHO 2012.*)
- ²⁵ Appropriate professions are defined by the Rulebook on the list of professional, academic and scientific titles ("Official Gazette of the RS", no. 30/07, 112/08, 72/09 and 81/10) as well as the Rulebook on specializations and narrower specializations of health workers and health associates ("Official Gazette of RS", number 10/13).